Implementation and Improvement Science - Interhnas

① know → do

Quality improvement - strenghtening services and improving care by ensuring safe, timely, effective, efficient, patient centred, equitable care

Project
Step 1) Study - qualitative interviews
2) Intervention -

. PODCAST

② healthcare should be =

My project ... ?

- efficient - will it make a service run more smoothly with less resources?
- effective - will it lead to better outcomes?
- safe - does it put anyone at risk?
- timely - is it possible to complete in the time frame?
- patient - centred - does it have patient's best interests at its core?
- equitable - will it benefit those who are most in need?

→ help provide a framework
→ measurable
→ HC quality is proportional to excellence
→ quality improvement: 1 of these domains

1) What are you going to improve and by how much?

2) How will you know if a change is an improvement? What is doable and reliable?

3) What changes can you make that will lead to improvement?

Plan, Do, Study, Act

Adapt
Adopt
Discard

Take into account → Innovation fatigue
→ Priorities

Institute for healthcare improvement

3 Understanding cost effectiveness and health economics

Compare 2 treatments
- Cost
- Outcome / Consequences

+ PODCAST

- Costs
  - direct
  - indirect
  - intangible

  effectiveness / cost

- Balance between cost / effectiveness

- Costs can be
  - fixed
  - variable

- Introduction to Prudent and value-based healthcare

  Choosing wisely — ↓ over medicalization

  ICHOM

  1) Public and prof. are equal partners through co-production
  2) Care for those in greatests health need first
  3) Do only what is needed and <u>do no harm</u>
  4) Reduce inappropriate variation through evidence-based approach

  Only do what you can do

Value Based health care → value $= \frac{\text{outcome}}{\text{cost}}$ that matters to patients of delivery the outcomes

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SECTION 2 - KNOWLEDGE

- PLAN, MONITOR, ANALYSE THE SITUATION

1) Identifying a problem
2) Tools and methods to understand the problem
3) What is a SMART aim?
4) Building a team
5) Strategies for change
6) The importance of PDSA cycles and audit cycles
7) Knowledge to action and behaviour change
8) Change is not easy.

- Change ← Realistic / Feasible | with available resources and time frame
- Smalls steps well done
- QI project – engage team members → improvements are known and sustainable

(1) How to Identify the problem –

What is happening // What should be happening

- Clinical Audits
- Incident reports
- Patient outcomes

- Issue
- Realise the problem
- Observations
- Planning stage

Understand the problem =

(1) Process mapping
SWOT Analysis
Pareto charts
Driver diagrams
Scatter diagram
Fishbone diagrams
The five whys

(1) Scheme to understand better the steps and what can be done to target better the issues

Pareto Chart 80/20

80 ↑ vital few

20 ↖ trivial many

7. Tree Diagram –

- Primary Driver
- Secondary Driver
- Change idea

Drivers (2-5) — Projects / Activities

Goal (clearly defined + measurable) → Primary driver ← Project

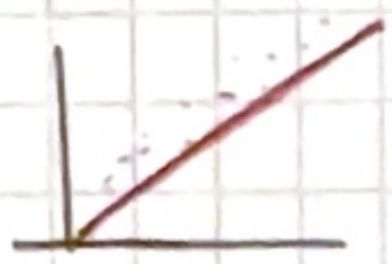
What is the problem we're addressing

What are the problems that cause the main problem?

What can you do?

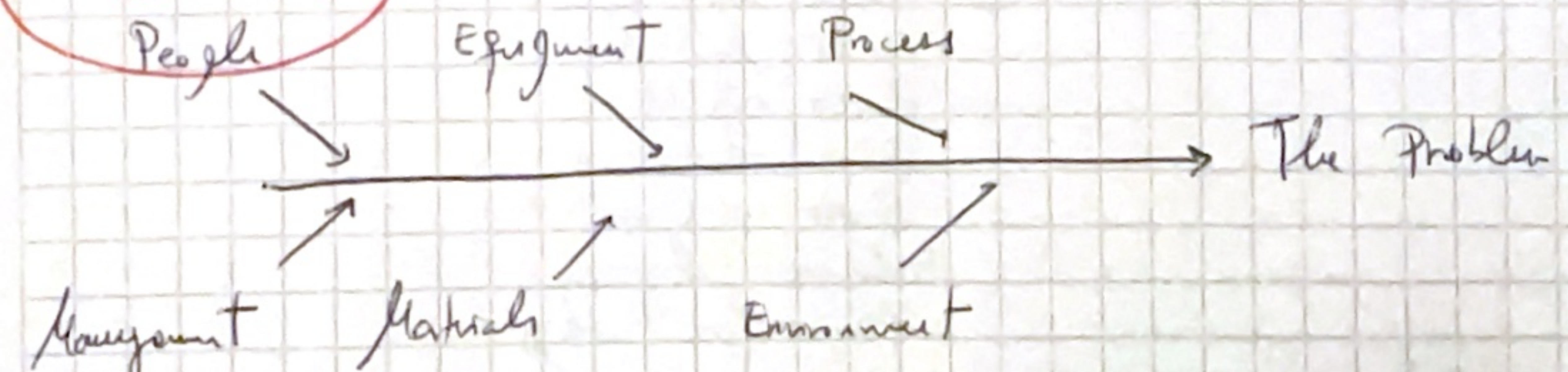
1) Define the goal
2) What things do we need to ~~achieve~~ improve to achieve our goal?
3) After brainstorming

Scatter diagrams that show the relationship and not causation.



Fishbone Diagram – Cause and effect

or **Ishikawa**



Creating a SMART aim

⇒ Robust method for thinking about your study

S specific
M measurable
A achievable
R relevant
T timely

Team

- Who will lead the project
- Who will be impacted by the project
- Who will provide the clinical and technical expertise
- Who will sponsor / support the project?

who – Managing the team
x
x
x
- How often will I contact them
- Methods
- Assess people's roles
- who is in charge of [illegible]

Strategies for Change

- Making a plan

PDSA and Audit Cycles

→ Measuring THROUGHOUT

Podcast

THE AUDIT CYCLE

- Good way to measure and monitor a project throughout

- PDSA Cycle = Another tool

Plan, Do, Study, Act

Knowledge to Action and Behaviour Change

Knowledge to Action Pipeline

Knowledge ⟷ Action

distance

Aware To	Accepted	Applicable	Able	Acted on	Agreed	Adhered

we research

how things work in practice

will people change the behaviours and make it sustainable

Behaviour Change Wheel

book

Susan Michie

helps in the design of intervention

1) hub - centre -
capability
opportunity
motivation

2) Intervention

3) Policy categories

Change

Start by small incremental

BACKGROUND

Rationale ← grant the case ← use data be clear

outline the benefit ← negative impact to avoid

how does it align with local or national agenda.

Sell the reasons it's a good idea

Identifying the problem
- is there a discrepancy between practice and the ideal/planned situation?
- Is there a clear reason for this?
- what is the problem? How severe is it? What are the consequences?
- Are there any factors that may have contributed to the problem?

Best Practice
- Demonstrate what is / what's not known
- Rethink - why it's crucial
- highlights what's not known.
- Shows that I know what is

Team / Stakeholders

Understand the Problem ← SWOT Analysis
Process map
Pareto Chart